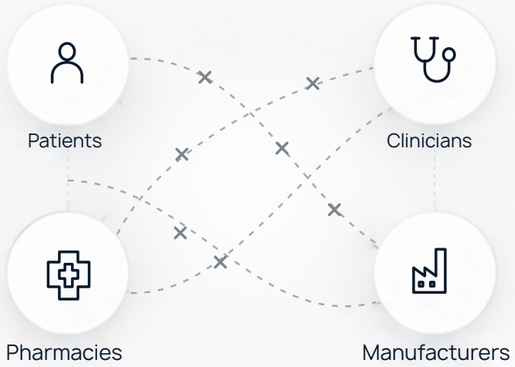
The next era of cannabinoid medicine.

BEFORE
A fragmented industry.



TODAY
pH7 connected infrastructure.

Convenience · Safety · Reliability



TOMORROW
PCCI™ Precision clinical intelligence.

Proposed patient-specific A.I. guidance with clinician oversight.



Distribution + intelligence = potential compounding advantage.

Patients → Medical Activity → Revenue → Structured Interactions → Improved Intelligence → Greater Adoption

An industry built on disconnected systems.

01
PATIENTS
Fragmented patient experience
- Limited treatment access
- Inconsistent quality of care
- High costs and poor continuity



02
CLINICIANS
Prescribing without precision
- Fragmented clinical evidence
- Limited prescribing confidence
- Inconsistent treatment outcomes



03
PHARMACIES
Disconnected fulfilment
- Fragmented supplier communication
- Limited stock visibility
- Inefficient inventory management



04
MANUFACTURERS
Limited market intelligence
- Disconnected patient demand
- Minimal clinical feedback
- Limited product performance data



THE CLINICAL CONFIDENCE GAP

88%

Unsure of an effective THC dose

89%

Unsure of an effective CBD dose

Published survey of 244 healthcare providers.

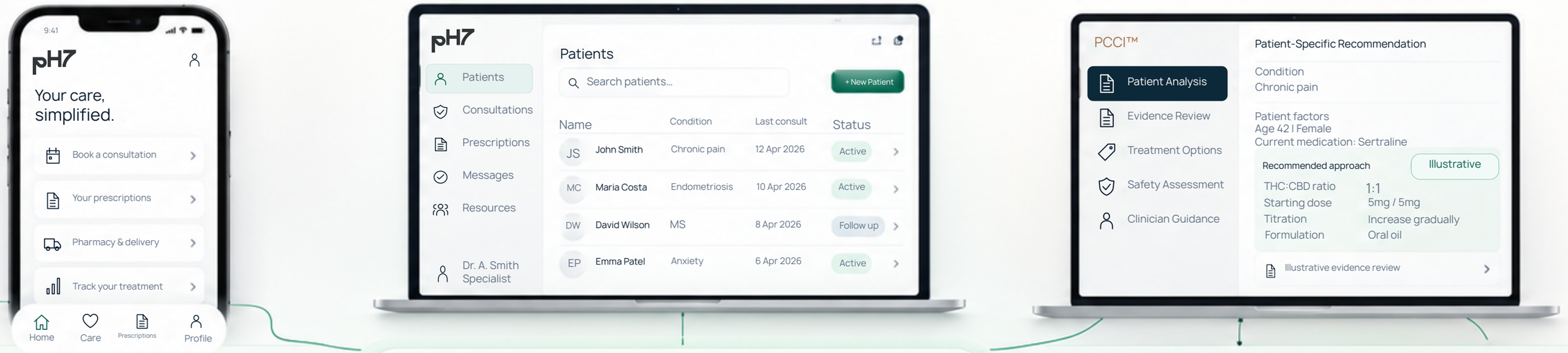
Kruger et al., 2022 · doi:10.1089/can.2021.0032. Survey: 244 physicians, one US health system, 2020.

One platform.
The entire ecosystem.

PATIENT ACCESS
Consultations · Prescriptions · Home delivery

CLINICAL DELIVERY
Independent clinicians. Constant workflows.

CLINICAL INTELLIGENCE
PCCI integration in development.



Patients — Access care
Clinicians — Deliver treatment

ONE CONNECTED INFRASTRUCTURE

Pharmacies — Dispense & fulfil
Manufacturers — Supply products
Regulators — Compliance & governance

PCCI™ — THE INTELLIGENCE LAYER
Evidence · Patient modelling · Safety · Clinical guidance

Clinical Activity → Structured Data → CCIE Evaluation → Improved Intelligence → Greater Platform Value

3.5+ million research records.
One clinician-reviewable assessment.

3.63M
research-source records
Publication · Registry · Product records

355,793
indexed publications
Company-reported publication index

1,000+
cannabinoid products
Reported product index;
count and status unverified.

PCCI™

Evidence harmonisation
↓
Patient-specific modelling
↓
Safety evaluation
↓
Therapeutic optimisation

PATIENT-SPECIFIC THERAPEUTIC ASSESSMENT

Illustrative output

Patient profile
Female | 42 years

Primary indication
Chronic neuropathic pain

Medical history
Fibromyalgia, sleep disturbance

Current medication
Gabapentin

Treatment objective
Pain management with daytime functionality

RECOMMENDED FORMULATION

Illustrative



CBD-dominant oral formulation
CBD:THC 20:1
Oral oil

Starting dose	5mg CBD once daily (PM)
Titration	Increase by 5mg every 3–7 days as tolerated
Safety	Interaction review required
Evidence	Illustrative evidence references
Product options	Illustrative product options

Clinician review required — Final decision at the discretion of the prescribing clinician.

Company-reported benchmarks, not clinical validation. Summary, 25 Oct 2026, pp. 4–5, 9. Rights pending; see appendix.

97.1% Agreement with published doses
Company-reported dose comparison

84/84 Safety tests passed
Designated safety-stop tests (company-reported).

80+ Career patents (reported)
Nearly 40 years in biomedical AI

Proposed clinical intelligence infrastructure
Proposed €500 / clinic / month

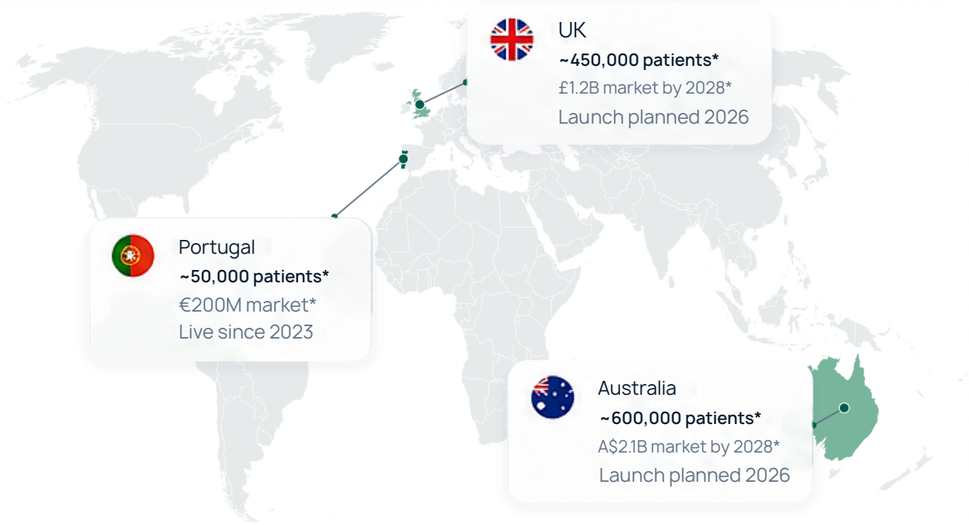
A global market.
Three entry points.

€50B+*
Global market by 2030
CAGR ~20% (2025–2030)*

3M+
Legacy patient estimate*

€8–12B
Unverified opportunity*

UK, AU & PT · Illustrative
Methodology unresolved.



MARKET ENTRY PLAN

Portugal	Lisbon delivery target	Nationwide delivery target
United Kingdom	East Midlands pilot	National rollout
Australia	Queensland first	National expansion

FURTHER EXPANSION

European opportunity
Regulated European markets

Additional markets
North America · LATAM
Selected Asia-Pacific markets

600k+
Reported AU patients*

20%+
Reported CAGR to 2030*

1,000+
Reported product count*

Regulatory pathways
Market-specific requirements apply in each jurisdiction

* Legacy estimates unverified. Market definitions, current sources and €8–12B methodology gap: see evidence appendix.

One infrastructure.
Multiple revenue engines.

TRANSACTION REVENUE

20% — Consultation commission
Modelled net fee: €5 of €25.

20% — Product revenue share
PT manufacturer wholesale basis.

CONNECTED
HEALTHCARE INFRASTRUCTURE

RECURRING & INTELLIGENCE REVENUE

€5K – €10K — Manufacturer Enterprise subscriptions
Proposed monthly tiers.

€500 — PCCI™ licensing
Proposed monthly clinic fee.

∞ — Research & data products
Proposed research & aggregated insights

01 Activity drives revenue → 02 Scale increases value → 03 Intelligence creates new markets

Terms vary: UK £1.50/order; AU fixed-fee draft. PCCI rights, research products and enterprise tiers remain proposed.

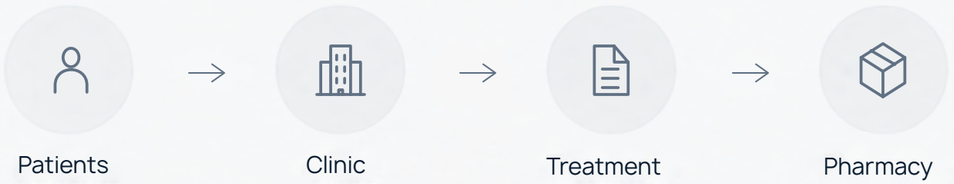
They built clinics.
We built the infrastructure.

Operating-model business comparison (UK)

TRADITIONAL OPERATORS

Clinic-centric model

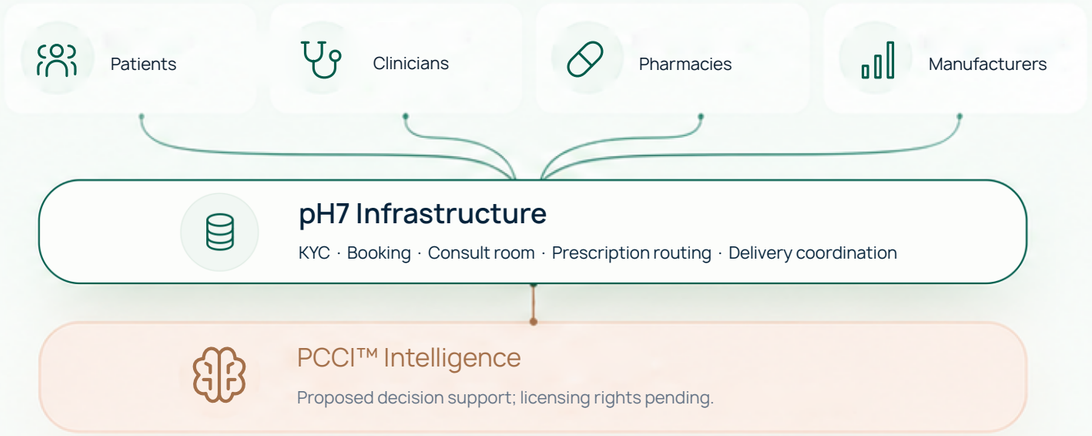
Clinic-led care with pharmacy partners.



UK market examples:
Alternaleaf • Curaleaf Clinic • Mamedica • Releaf

Infrastructure + intelligence model

Independent providers on one platform.



01 Asset-light infrastructure
Scale without owning clinics, pharmacies or cultivation.

02 Multi-sided network

03 PCCI™ intelligence
Independent clinical licensing potential.

04 International architecture
Designed for multiple regulated jurisdictions.

The infrastructure is built. Now we scale.

€2.50
Reported cost per install
Portugal · attribution unverified

5×
Reported campaign ROI
Apr 2025 · basis unverified

€300K+
Annual revenue potential
Management pipeline; not recognised revenue.

100+
Reported active patients
Date / cohort unverified

DIGITAL ACQUISITION ADVANTAGE

€15K/month
Reported Google advertising capacity
Capacity unverified; not actual spend.

120K+
Reported MedBud visits
Proposed UK channel; traffic period unverified.

Google certification documented.
Scope requires confirmation.
Public Rx advertising prohibited.

FROM INVESTMENT TO SCALE

Growth capital → Patient acquisition → Clinical activity → Revenue → Scale

INTERNATIONAL READINESS

PORTUGAL
Operating
Established commercial network

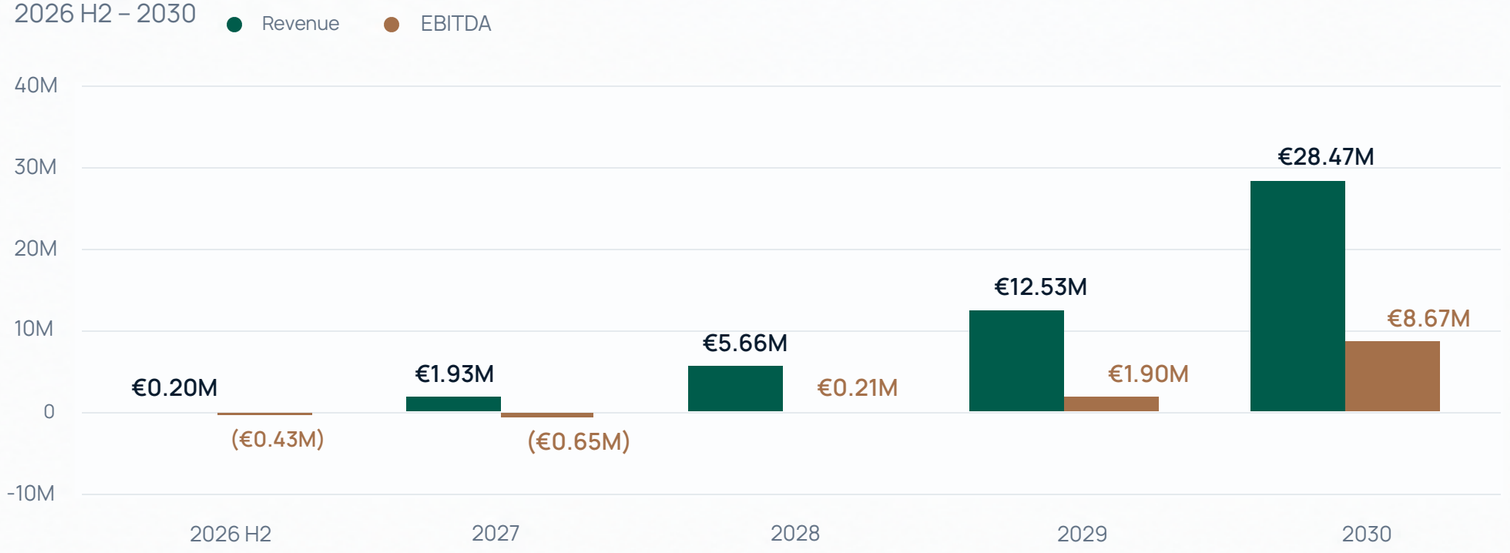
UNITED KINGDOM
Launch infrastructure
Clinical, pharmacy & distribution relationships

AUSTRALIA
Early activation
Clinical onboarding & pharmacy terms in progress.

Built for scale.
Designed for operating leverage.

Reconciled to the original model. Bull case; calendar periods, with July–December only in 2026.

PROJECTED REVENUE & EBITDA · BULL
2026 H2 – 2030



ANNUALISED EXIT REVENUE SCENARIOS

BEAR CASE	BASE CASE	BULL CASE
€7.14M	€26.35M	€61.76M
Slower market adoption and enterprise uptake	Current commercial plan execution	Faster international growth, enterprise adoption and platform services

June 2031 monthly revenue ×12. F01: scenario cells BI24.

2030 KEY METRICS

€8.67M
Projected EBITDA

30.4%
Projected EBITDA margin

Source: F01 Bull Case, AR:BC, rows 23/31. Calendar year 2030.

KEY MODEL ASSUMPTIONS

01 **Patient acquisition**
€51 initial patient CAC
Scales to €127.50 under model assumptions

02 **Transaction revenue**
€5 net per consultation; 20% modelled product share

03 **Enterprise subscriptions**
€5.85K–€11.7K / month

04 **Platform services**
€10K / month in bull case

Model: €6.5M funding in Jul 2026/2027/2029. Proposed €2.4M and timing remain unreconciled.

The expertise to build it.
The experience to scale it.

OK

Oska Keene
Founder & CEO

3
priority markets

Portugal · UK · Australia

RS

Ron Smith
Chief Technology Officer

18+
years in technology

Former fintech CTO
Engineering teams of up to 150

SM

Stuart Marsh
Australia Lead

25 years
in commercial innovation

Former Chief Commercial Officer, Cannim

CE

Chris Elliott
Clinical Liaison

UK & Australian
clinical operations

Medical cannabis clinic launches & operations

JM

Josh Munk
Commercial & Partnerships

Nearly 30 years
in cannabis

Cultivation to retail
US & international markets

ZA

Dr Zubair Ahmed
Clinical Advisor

Professor
of Neuroscience

Neurotrauma & CNS regeneration
MRC · NIHR funding
University of Birmingham

SIX PEOPLE.
FIVE CRITICAL DISCIPLINES.

Technology	Biomedical AI	Clinical Operations	Regulatory Markets	Commercialisation

The infrastructure is built. The opportunity is now.

Growth capital for international commercialisation.

€400,000
Proposed convertible bridge

20%
Proposed conversion discount
At the next qualifying financing (subject to revised documentation).

€2M
Targeted institutional financing
Planned for Q1 2027

WHY NOW

01 REGULATION IS ESTABLISHED
Prescribing pathways across key international markets.

02 THE INFRASTRUCTURE EXISTS
Technology, clinicians and pharmacy networks in place.

03 THE INTELLIGENCE ADVANTAGE
Potential for a new clinical prescribing standard.

USE OF FUNDS | €400,000



40%	25%	20%	10%	5%
€160,000	**€100,000**	**€80,000**	**€40,000**	**€20,000**
UK launch and regulatory finalisation.	Australia launch and partner activation.	Product and clinical team expansion.	Marketing and patient acquisition.	Working capital and contingencies.

NEXT 12 MONTHS

Q4 2026
Bridge financing
€400,000 bridge for commercial activation

Q1 2027
Institutional financing
Target €2M for UK expansion and Australian activation

Q2 – Q3 2027
Commercial scale
Patient acquisition, enterprise partnerships & PCCI licensing

€400K proposal vs €200K draft note; deck €2.4M vs model €6.5M. Financing reconciliation: see appendix.

SEE EVIDENCE APPENDIX